

FAX

Center of Revitalizing Psychiatry
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TO:

Name: Carhall Lawfirm

Fax Number: (941) 914-9629

of Pages: 29

(including cover sheet)

FROM:

Name: Center of Revitalizing Psychiatry

Fax Number: (941) 677-3367

Subject: Re: Kohn, Michelle

Message:

Please see the attached medical record request for Michelle, Kohn. The client has paid for the request as of 02/19/24. this is for invoice Number 1000173MC If you have any questions please feel free to contact our office.

Kind Regards,

Meredith Cornett

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 241584953 Encounter Date: 07/20/2021

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient has given verbal consent that he understands the limitations of telehealth and consents to proceed with treatment through this mode, zoom.

This is a 33 year old woman with a history of ADHD inattentive type. Medication Management visit.

Today Michelle reports "I am feeling pretty well, functioning well at work and at home".

ADHD: "well controlled since increasing the morning dose of Adderall".

ANXIETY: "present, but it is well controlled". "a lot better since self employee".

COMPLIANCE WITH MEDICATIONS: Yes. SIDE EFFECTS: Denies.

Sleep: "good, 7-8 hours per night" Taking Melatonin once in while.

Appetite: "fine"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

Florida PMP: overdose risk score 0/999. 6/14/21 Adderall 20mg 75/30

History Of Present Illness:

Self employee working from home. She lives with boyfriend and daughter.

Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY:

Past Psychiatric History: Yes. ADHD

Psych. Hospitalizations: denies

Past suicidal ideation or behavior: denies

Past violent ideations or behavior: denies

List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies

SUBSTANCE ABUSE HISTORY:

Substance Abuse History: Denied.

Allergies:

No known allergies

Current Medications:

none

Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon no later than 2pm); Qty: 75; Refills: 0

Vistaril 50 MG Oral Capsule; Take 1 capsule orally 3 times a day PRN for anxiety and or sleep; Qty: 90; Refills: 0

OBJECTIVE:

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 241584953 Encounter Date: 07/20/2021

Encounter Type: Office Visit

Vital Signs: Height: 63.00 in
 Weight: 125.00 lbs
 BMI: 22.14
 Blood Pressure: 110/75 mmHg
 B/P Position: Sitting
 Pulse: 92 beats/min

Objective Notes: MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/
 Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "good". Affect: Stable. Thought
 Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and
 JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies
 suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language:
 Intact. Orientation: Ox4. Time. Place. Person. Situation.
 Short Term Memory: Intact Long Term Memory: Intact. Ability of Retrieving information and rapid forgetting.
 Ability of Encoding information Intact Concentration/Attention: Intact . Judgment /Insight: Intact

ASSESSMENT:

Diagnosis: ICD-10 Codes:
 1)F902; Attention-deficit hyperactivity disorder, combined
 2)F411; Generalized anxiety disorder

PROBLEM / Problem/Condition: Established.
CONDITION: Status: Stable.

PLAN:

Procedures: 1) 99214; E&M, moderate problem
 2) 90833; Psychotherapy 16-37 min add on code

Procedure Notes: Michelle presented calm, pleasant and optimistic. ADHD symptoms had improved since titrating Adderall dose.

Agreed to continue Adderall 20mg 1.5 tablets in the am and 1 tablet in the afternoon. Continue Vistaril 50mg
 TID PRN for anxiety

Medications: She will continue monitoring weight, blood pressure and heart rate at home.
 Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon
 no later than 2pm); Qty: 75; Refills: 0
 Vistaril 50 MG Oral Capsule; Take 1 capsule orally 3 times a day PRN for anxiety and or sleep; Qty: 90;
 Refills: 0

Care Plan: The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia,
 tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out
 the day.

Advised the patient to refrain from drug or alcohol while taking medications

Advised the patient to take the medication with food if GI upset occurs .

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 241584953 Encounter Date: 07/20/2021

Encounter Type: Office Visit

Patient Instructions: For any serious side effects recommended that patient stop the medication immediately.

Next Appointment: 3 months in office

LEVEL of MDM/ MODERATE

Counseling:

1. Stable 2 or more illnesses
2. Reviewed previous note, eforcse
3. Prescription drug management

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Counseling/psychotherapy

Supportive therapy was provided.

Psychoeducation therapy was provided.

Psychodynamic therapy was provided.

Patient shows understanding of counseling.

Electronically signed by: Eduardo LaRosa, ARNP

_____ Date: _____

[Provider]: Eduardo La Rosa, APRN

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 239419076 Encounter Date: 06/08/2021

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient has given verbal consent that he understands the limitations of telehealth and consents to proceed with treatment through this mode, zoom.

This is a 33 year old woman with a history of ADHD inattentive type. Medication Management visit.

Today Michelle reports "I am feeling and doing well, my medication is working pretty well. Work is very busy, a little crazy. I am in the process of switching full time to real state and not do property management anymore".

ADHD: "well controlled, but sometimes it is little harder. I take my morning dose of Adderall around 8-9am but I feel it is wearing off around 11:30-12, I take my second dose around 2 with my lunch, no issues in the afternoon"

ANXIETY: "present, but it is well controlled". Anxiety is related to work stress".

COMPLIANCE WITH MEDICATIONS: Yes. SIDE EFFECTS: Denies.

Sleep: "good, 7-8 hours per night" Taking Melatonin once in while.

Appetite: "fine"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

Florida PMP: overdose risk score 0/999. 5/15/21 Adderall 20mg 60/30.

History Of Present Illness:

She works full time as property manager and real state agent. She lives with boyfriend and daughter. Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY:

Past Psychiatric History: Yes. ADHD
Psych. Hospitalizations: denies
Past suicidal ideation or behavior: denies
Past violent ideations or behavior: denies
List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies

SUBSTANCE ABUSE HISTORY:

Substance Abuse History: Denied.

Allergies:

No known allergies

Current Medications:

none
Adderall 20 MG Oral Tablet; Take 1 tablet orally 2 times a day 1 tablet in the am, 1 tablet in the afternoon no later than 2pm; Qty: 60; Refills: 0

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 239419076 Encounter Date: 06/08/2021

Encounter Type: Office Visit

Vistaril 50 MG Oral Capsule; Take 1 capsule orally 3 times a day PRN for anxiety and or sleep; Qty: 90; Refills: 0

OBJECTIVE:

Objective Notes:

MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/ Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "good". Affect: Stable. Thought Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language: Intact. Orientation: Ox4. Time. Place. Person. Situation. Short Term Memory: Intact Long Term Memory: Intact. Ability of Retrieving information and rapid forgetting. Ability of Encoding information Intact Concentration/Attention: Intact . Judgment /Insight: Intact

ASSESSMENT:

Diagnosis:

ICD-10 Codes:

- 1)F902; Attention-deficit hyperactivity disorder, combined
- 2)F411; Generalized anxiety disorder

**PROBLEM /
CONDITION:**

Problem/Condition: Established.
Status: Stable.

PLAN:

Procedures:

1) 0; 99214+90833 (psychotherapy 16-37min)

Procedure Notes:

Michelle presented calm, pleasant and optimistic. Struggling with ADHD symptoms in the am .

Agreed to increase Adderall 20mg 1.5 tablets in the am and 1 tablet in the afternoon. Continue Vistaril 50mg TID PRN for anxiety

Medications:

She will start monitoring weight, blood pressure and heart rate at home.

Adderall 20 MG Oral Tablet; Take tablets orally daily (Take 1.5 tablets in the am and 1 tablet in the afternoon no later than 2pm); Qty: 75; Refills: 0

Care Plan:

The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia, tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out the day.

Advised the patient to refrain from drug or alcohol while taking medications

Advised the patient to take the medication with food if GI upset occurs .

Patient Instructions:

For any serious side effects recommended that patient stop the medication immediately.

LEVEL of MDM/

MODERATE

Counseling:

1. Stable 2 or more illnesses
2. Reviewed previous note, eforce

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 239419076 Encounter Date: 06/08/2021

Encounter Type: Office Visit

3. Prescription drug management

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Counseling/psychotherapy

Supportive therapy was provided.

Psychoeducation therapy was provided.

Psychodynamic therapy was provided.

Patient shows understanding of counseling.

Electronically signed by: Eduardo LaRosa, ARNP

_____ Date: _____

[Provider]: Eduardo La Rosa, APRN

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 230305243 Encounter Date: 12/10/2020

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient has given verbal consent that he understands the limitations of telehealth and consents to proceed with treatment through this mode, zoom.

This is a 33 year old woman with a history of ADHD inattentive type. Medication Management visit.

Today Michelle reports "I am feeling well, my medication is working pretty well. Work is going well, very busy. Everything is good at home".

ADHD: "well controlled"

ANXIETY: "present, but it is well controlled". Anxiety is mainly caused by work stress.

Sleep: "good, 7-8 hours per night" Taking Melatonin once in while.

Appetite: "fine"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

Florida PMP: overdose risk score 0/999. 0/999. 11/1/20 Adderall 20mg 60/30.

History Of Present Illness:

She works full time as property manager and real state agent. She lives with boyfriend and 7 year old daughter. Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY:

Past Psychiatric History: Yes. ADHD
Psych. Hospitalizations: denies
Past suicidal ideation or behavior: denies
Past violent ideations or behavior: denies
List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies

SUBSTANCE ABUSE HISTORY:

Substance Abuse History: Denied.

Allergies:

No known allergies

Current Medications:

none
Adderall 20 MG Oral Tablet; Take 1 tablet orally 2 times a day 1 tablet in the am, 1 tablet in the afternoon no later than 2pm; Qty: 60; Refills: 0

OBJECTIVE:

Objective Notes:

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 230305243 Encounter Date: 12/10/2020

Encounter Type: Office Visit

MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/ Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "good". Affect: Stable. Thought Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language: Intact. Orientation: Ox4. Time. Place. Person. Situation. Short Term Memory: Intact Long Term Memory: Intact. Ability of Retrieving information and rapid forgetting. Ability of Encoding information Intact Concentration/Attention: Intact . Judgment /Insight: Intact

ASSESSMENT:

Diagnosis:

ICD-10 Codes:

1) F902; Attention-deficit hyperactivity disorder, combined

PLAN:

Procedures:

1) 0; 99214+90833 (psychotherapy 16-37min)

Procedure Notes:

Michelle presented calm, pleasant, optimistic and cooperative. She reports ADHD symptoms are well controlled with Adderall at current dose. Anxiety has increased mainly related to work stress.

Medications:

We agreed to continue Adderall 20mg 1 tablet twice a day. We will start Vistaril 25mg TID PRN for anxiety Adderall 20 MG Oral Tablet; Take 1 tablet orally 2 times a day 1 tablet in the am, 1 tablet in the afternoon no later than 2pm; Qty: 60; Refills: 0

Vistaril 25 MG Oral Capsule; Take 1 capsule orally 3 times a day PRN for anxiety; Qty: 90; Refills: 0

Care Plan:

The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia, tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out the day.

Advised the patient to refrain from drug or alcohol while taking medications

Advised the patient to take the medication with food if GI upset occurs .

Patient Instructions:

For any serious side effects recommended that patient stop the medication immediately.

TIME SPENT IN

Next Appointment: 2-3 Months.

COUNSELING:

Greater than 50% of time spent in counseling.

Counseling/psychotherapy time 16 min

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Patient shows understanding of counseling

Supportive therapy was provided.

Psychoeducation therapy was provided.

Psychodynamic therapy was provided.

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 230305243 Encounter Date: 12/10/2020

Encounter Type: Office Visit

Duration of visit: 20min.

_____ Date: _____

[Provider]: Eduardo La Rosa, APRN

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 234447703 Encounter Date: 03/04/2021

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient has given verbal consent that he understands the limitations of telehealth and consents to proceed with treatment through this mode, zoom.

This is a 33 year old woman with a history of ADHD inattentive type. Medication Management visit.

Today Michelle reports "I am feeling pretty well, my medication is working pretty well. Work is very busy, a lot going on at work, which made me a little more anxious".

ADHD: "well controlled"

ANXIETY: "present, but it is well controlled". Anxiety is related to work stress".

Sleep: "good, 7-8 hours per night" Taking Melatonin once in while.

Appetite: "fine"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

Florida PMP: overdose risk score 0/999. 2/12/21 Adderall 20mg 60/30.

History Of Present Illness:

She works full time as property manager and real state agent. She lives with boyfriend and daughter.

Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY:

Past Psychiatric History: Yes. ADHD

Psych. Hospitalizations: denies

Past suicidal ideation or behavior: denies

Past violent ideations or behavior: denies

List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies

SUBSTANCE ABUSE HISTORY:

Substance Abuse History: Denied.

Allergies:

No known allergies

Current Medications:

none

Adderall 20 MG Oral Tablet; Take 1 tablet orally 2 times a day 1 tablet in the am, 1 tablet in the afternoon no later than 2pm; Qty: 60; Refills: 0

OBJECTIVE:

Objective Notes:

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 234447703 Encounter Date: 03/04/2021

Encounter Type: Office Visit

MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/ Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "good". Affect: Stable. Thought Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language: Intact. Orientation: Ox4. Time. Place. Person. Situation. Short Term Memory: Intact Long Term Memory: Intact. Ability of Retrieving information and rapid forgetting. Ability of Encoding information Intact Concentration/Attention: Intact . Judgment /Insight: Intact

ASSESSMENT:

Diagnosis:

ICD-10 Codes:

1) F902; Attention-deficit hyperactivity disorder, combined

PLAN:

Procedures:

1) 0; 99214+90833 (psychotherapy 16-37min)

Procedure Notes:

Michelle presented calm, pleasant, optimistic and cooperative. She reports ADHD symptoms are well controlled with Adderall at current dose. Anxiety is still present mainly related to work stress. Vistaril 25mg 2 tabs was not helpful, requested to increase dose.

We agreed to continue Adderall 20mg 1 tablet twice a day. Increase Vistaril 50mg TID PRN for anxiety

Medications:

Sent Adderall rx for 3/12/21. She will call for refill on April and May

Adderall 20 MG Oral Tablet; Take 1 tablet orally 2 times a day 1 tablet in the am, 1 tablet in the afternoon no later than 2pm; Qty: 60; Refills: 0

Vistaril 50 MG Oral Capsule; Take 1 capsule orally 3 times a day PRN for anxiety and or sleep; Qty: 90; Refills: 0

Care Plan:

The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia, tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out the day.

Advised the patient to refrain from drug or alcohol while taking medications

Advised the patient to take the medication with food if GI upset occurs .

Patient Instructions:

For any serious side effects recommended that patient stop the medication immediately.

TIME SPENT IN

Next Appointment: 3 Months.

COUNSELING:

Greater than 50% of time spent in counseling.

Counseling/psychotherapy time 16 min

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 234447703 Encounter Date: 03/04/2021

Encounter Type: Office Visit

Patient shows understanding of counseling

Supportive therapy was provided.

Psychoeducation therapy was provided.

Psychodynamic therapy was provided.

Duration of visit: 20min.

_____ Date: _____

[Provider]: Eduardo La Rosa, APRN

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 222735402 Encounter Date: 07/09/2020

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient has given verbal consent that he understands the limitations of telehealth and consents to proceed with treatment through this mode

This is a 32 year old woman with a history of ADHD inattentive type. Medication Management visit.

Today Michelle reports "I am feeling pretty well. Medication is working pretty well specially in the am, in the afternoon I struggle a little".

ADHD: "improving" Reports: careless mistakes and/or lack of attention to details, difficulty sustaining attention in tasks, does not seem to listen when spoken directly, failure to follow through on tasks, poor organization, losing things for task, easily distracted, forgetful in daily activities, avoids tasks that sustain/require effort, procrastinate, fidgets, squirms, leaves seat when expected to stay seated, restlessness, often on the go / acts as if driven by a motor, talks excessively, blurts out answers before questions have been completed, difficulty awaiting turn / interrupts or intrudes on others (butts into conversation or game). all symptoms continue improving

ANXIETY: "well controlled" Reports: worry, fears, apprehension, irritability, frustrated, angry, difficulty concentrating, racing and recurrent thoughts are a lot better

Sleep: "good, 7-8 hours per night. no taking trazodone. Taking Melatonin once or twice per week when needed for sleep".

Appetite: "fine"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

Florida PMP: overdose risk score 0/999. 6/11/20 Adderall 20mg 45/30

History Of Present Illness:

She works full time as property manager and real state agent. She lives with boyfriend and 7 year old daughter. Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY:

Past Psychiatric History: Yes. ADHD

Psych. Hospitalizations: denies

Past suicidal ideation or behavior: denies

Past violent ideations or behavior: denies

List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies

Substance Abuse History: Denied.

SUBSTANCE ABUSE HISTORY:

Allergies:

No known allergies

Current Medications:

none

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 222735402 Encounter Date: 07/09/2020

Encounter Type: Office Visit

Adderall 20 MG Oral Tablet; Take 1.5 tablets orally daily Take 1 tablet in the am with food. Take 1/2 tablet in the afternoon.; Qty: 45; Refills: 0

OBJECTIVE:

Objective Notes:

MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/ Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "good". Affect: Stable. Thought Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language: Intact. Orientation: Ox4. Time. Place. Person. Situation. Short Term Memory: Intact Long Term Memory: Intact. Ability of Retrieving information and rapid forgetting. Ability of Encoding information Intact Concentration/Attention: Intact . Judgment /Insight: Intact

ASSESSMENT:

Diagnosis:

ICD-10 Codes:

1)F902; Attention-deficit hyperactivity disorder, combined

PLAN:

Procedures:

1) 0; 99214+90833 (psychotherapy 16-37min)

Procedure Notes:

Michelle presented calm, pleasant, optimistic and cooperative.

She reports ADHD symptoms have improved since increasing Adderall dose to 20mg in the am. However, she struggles in the afternoon.

We agreed to increase Adderall 20mg 1 tablet twice a day.

Medications:

She will continue melatonin as needed for sleep.

Adderall 20 MG Oral Tablet; Take 1 tablet orally 2 times a day 1 tablet in the am, 1 tablet in the afternoon no later than 2pm; Qty: 60; Refills: 0

Care Plan:

The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia, tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out the day.

Advised the patient to refrain from drug or alcohol while taking medications

Advised the patient to take the medication with food if GI upset occurs .

Patient Instructions:

For any serious side effects recommended that patient stop the medication immediately.

TIME SPENT IN

Next Appointment: 1 Month.

COUNSELING:

Greater than 50% of time spent in counseling.

Counseling/psychotherapy time 16 min

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 222735402 Encounter Date: 07/09/2020

Encounter Type: Office Visit

Patient shows understanding of counseling

Supportive therapy was provided.

Psychoeducation therapy was provided.

Psychodynamic therapy was provided.

Duration of visit: 20min.

_____ Date: _____

[Provider]: Eduardo La Rosa, APRN

Center of Revitalizing Psychiatry
2033 Wood Street, Suite 220
Sarasota, FL 34237
Telephone 941-677-3366
Fax 941-677-3367

To:
Carhall Lawfirm
MDA Records Retrieval
Fax: 941-914-9629

Re: Medical Records Request

Invoice Number: 1000173MC

PT: Kohn, Michelle

DATE	SERVICE RENDERED	CHARGE	UNITS	BALANCE
1/16/24	Medical Records Fee	\$25.00	1	\$25.00
	Pages (\$1 per page)	\$48.00	48	48.00

TOTAL AMOUNT DUE \$73.00

**Make payable to: Center Of Revitalizing Psychiatry
or call the office with a credit card number
Tax ID # 043774404**

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 227144659 Encounter Date: 10/08/2020

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient has given verbal consent that he understands the limitations of telehealth and consents to proceed with treatment through this mode, zoom.

This is a 32 year old woman with a history of ADHD inattentive type. Medication Management visit.

Today Michelle reports "I am feeling well, my medication is working pretty. I am doing pretty well at work and in my personal life. I am not feeling anxious at all"

ADHD: "well controlled"

ANXIETY: "well controlled"

Sleep: "good, 7-8 hours per night" Taking Melatonin once in while.

Appetite: "fine"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

Florida PMP: overdose risk score 0/999. 9/10/20 Adderall 20mg 60/30

History Of Present Illness:

She works full time as property manager and real state agent. She lives with boyfriend and 7 year old daughter. Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY:

Past Psychiatric History: Yes. ADHD

Psych. Hospitalizations: denies

Past suicidal ideation or behavior: denies

Past violent ideations or behavior: denies

List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies

SUBSTANCE ABUSE HISTORY:

Substance Abuse History: Denied.

Allergies:

No known allergies

Current Medications:

none

Adderall 20 MG Oral Tablet; Take 1 tablet orally 2 times a day 1 tablet in the am, 1 tablet in the afternoon no later than 2pm; Qty: 60; Refills: 0

OBJECTIVE:

Objective Notes:

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 227144659 Encounter Date: 10/08/2020

Encounter Type: Office Visit

MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/ Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "good". Affect: Stable. Thought Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language: Intact. Orientation: Ox4. Time. Place. Person. Situation. Short Term Memory: Intact Long Term Memory: Intact. Ability of Retrieving information and rapid forgetting. Ability of Encoding information Intact Concentration/Attention: Intact . Judgment /Insight: Intact

ASSESSMENT:

Diagnosis:

ICD-10 Codes:

1) F902; Attention-deficit hyperactivity disorder, combined

PLAN:

Procedures:

1) 0; 99214+90833 (psychotherapy 16-37min)

Procedure Notes:

Michelle presented calm, pleasant, optimistic and cooperative. She reports ADHD symptoms are well controlled with Adderall at current dose. She is no longer feeling anxious.

We agreed to continue Adderall 20mg 1 tablet twice a day.

Medications:

We agreed to follow up in December. Sent rx for October, she will call for refill in November.

Adderall 20 MG Oral Tablet; Take 1 tablet orally 2 times a day 1 tablet in the am, 1 tablet in the afternoon no later than 2pm; Qty: 60; Refills: 0

Care Plan:

The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia, tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out the day.

Advised the patient to refrain from drug or alcohol while taking medications

Advised the patient to take the medication with food if GI upset occurs .

Patient Instructions:

For any serious side effects recommended that patient stop the medication immediately.

TIME SPENT IN

Next Appointment: 2 Months.

COUNSELING:

Greater than 50% of time spent in counseling.

Counseling/psychotherapy time 16 min

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Patient shows understanding of counseling

Supportive therapy was provided.

Psychoeducation therapy was provided.

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 227144659 Encounter Date: 10/08/2020

Encounter Type: Office Visit

Psychodynamic therapy was provided.

Duration of visit: 20min.

_____ Date: _____

[Provider]: Eduardo La Rosa, APRN

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 224074008 Encounter Date: 08/06/2020

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient has given verbal consent that he understands the limitations of telehealth and consents to proceed with treatment through this mode

This is a 32 year old woman with a history of ADHD inattentive type. Medication Management visit.

Today Michelle reports "I am feeling pretty well. My medication is working pretty. I just moved to a new house, I am happy, painting. I am exhausted but I am happy"

ADHD: "well controlled" Reports: careless mistakes and/or lack of attention to details, difficulty sustaining attention in tasks, does not seem to listen when spoken directly, failure to follow through on tasks, poor organization, losing things for task, easily distracted, forgetful in daily activities, avoids tasks that sustain/require effort, procrastinate, fidgets, squirms, leaves seat when expected to stay seated, restlessness, often on the go / acts as if driven by a motor, talks excessively, blurts out answers before questions have been completed, difficulty awaiting turn / interrupts or intrudes on others (butts into conversation or game). all symptoms continue improving

ANXIETY: "well controlled" Reports: worry, fears, apprehension, irritability, frustrated, angry, difficulty concentrating, racing and recurrent thoughts are a lot better

Sleep: "good, 7-8 hours per night. no taking trazodone. Taking Melatonin once or twice per week when needed for sleep".

Appetite: "fine"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

Florida PMP: overdose risk score 0/999. 0/999. 7/9/20 Adderall 20mg 60/30.

History Of Present Illness:

She works full time as property manager and real state agent. She lives with boyfriend and 7 year old daughter. Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY:

Past Psychiatric History: Yes. ADHD

Psych. Hospitalizations: denies

Past suicidal ideation or behavior: denies

Past violent ideations or behavior: denies

List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies

Substance Abuse History: Denied.

SUBSTANCE ABUSE HISTORY:

Allergies:

No known allergies

Current Medications:

none

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 224074008 Encounter Date: 08/06/2020

Encounter Type: Office Visit

Adderall 20 MG Oral Tablet; Take 1.5 tablets orally daily Take 1 tablet in the am with food. Take 1/2 tablet in the afternoon.; Qty: 45; Refills: 0

Adderall 20 MG Oral Tablet; Take 1 tablet orally 2 times a day 1 tablet in the am, 1 tablet in the afternoon no later than 2pm; Qty: 60; Refills: 0

OBJECTIVE:

Objective Notes:

MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/ Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "good". Affect: Stable. Thought Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language: Intact. Orientation: Ox4. Time. Place. Person. Situation. Short Term Memory: Intact Long Term Memory: Intact. Ability of Retrieving information and rapid forgetting. Ability of Encoding information Intact Concentration/Attention: Intact . Judgment /Insight: Intact

ASSESSMENT:

Diagnosis:

ICD-10 Codes:

1)F902; Attention-deficit hyperactivity disorder, combined

PLAN:

Procedures:

1) 0; 99214+90833 (psychotherapy 16-37min)

Procedure Notes:

Michelle presented calm, pleasant, optimistic and cooperative. She reports ADHD symptoms are well controlled since increasing dose to 20mg BID.

We agreed to continue Adderall 20mg 1 tablet twice a day.

Medications:

Sent rx for August 8th and September 7th

Adderall 20 MG Oral Tablet; Take 1 tablet orally 2 times a day 1 tablet in the am, 1 tablet in the afternoon no later than 2pm; Qty: 60; Refills: 0

Adderall 20 MG Oral Tablet; Take 1 tablet orally 2 times a day 1 tablet in the am, 1 tablet in the afternoon no later than 2pm; Qty: 60; Refills: 0

Care Plan:

The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia, tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out the day.

Advised the patient to refrain from drug or alcohol while taking medications

Advised the patient to take the medication with food if GI upset occurs .

Patient Instructions:

For any serious side effects recommended that patient stop the medication immediately.

TIME SPENT IN

Next Appointment: 2 Months.

COUNSELING:

Greater than 50% of time spent in counseling.

Counseling/psychotherapy time 16 min

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 224074008 Encounter Date: 08/06/2020

Encounter Type: Office Visit

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Patient shows understanding of counseling

Supportive therapy was provided.

Psychoeducation therapy was provided.

Psychodynamic therapy was provided.

Duration of visit: 20min.

_____ Date: _____

[Provider]: Eduardo La Rosa, APRN

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 220761427 Encounter Date: 05/26/2020

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient's identity has been confirmed. Patient has given verbal consent that he understands the limitations of telehealth and consents to proceed with treatment through this mode

This is a 32 year old woman with a history of ADHD inattentive type. Evaluation for medication treatment.

ADHD: Endorses: careless mistakes and/or lack of attention to details, difficulty sustaining attention in tasks, does not seem to listen when spoken directly, failure to follow through on tasks, poor organization, losing things for task, easily distracted, forgetful in daily activities, avoids tasks that sustain/require effort, procrastinate, fidgets, squirms, leaves seat when expected to stay seated, restlessness, often on the go / acts as if driven by a motor, talks excessively, blurts out answers before questions have been completed, difficulty awaiting turn / interrupts or intrudes on others (butts into conversation or game).

ANXIETY: Endorses: worry, fears, apprehension, irritability, frustrated, angry, difficulty concentrating, racing and recurrent thoughts "especially at night".

Denies depressed mood "I feel I am failing as an employee and as a mother, but I do not feel depressed".

Denies symptoms of mania or hypomania, denies psychosis. Denies SI/HI/I/P.

Sleep: "problems falling asleep, due to racing thoughts. I tried to sleep at least 8 hours per night".

Appetite: "fine"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

Florida PMP: patient found, no prescriptions found.

History Of Present Illness:

She works full time as property manager and real state agent. She lives with boyfriend and 7 year old daughter. Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. In the last couple months, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY:

Past Psychiatric History: Yes. ADHD

Psych. Hospitalizations: denies

Past suicidal ideation or behavior: denies

Past violent ideations or behavior: denies

List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies

SUBSTANCE ABUSE HISTORY:

Substance Abuse History: Denied.

Medical History:

The patient does not have a PCP and was encouraged to get a PCP

There is no history of medical condition relevant to the presenting problem.

Surgical History:

Left knee surgery 2003

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 220761427 Encounter Date: 05/26/2020

Encounter Type: Office Visit

Gynecological History: LMP 5/12/2020. BC - IUD.

Allergies: No known allergies

Current Medications: none

PSYCH FAMILY Family history of mental illness: denies

HISTORY: Family history of Substance Abuse: denies

SOCIAL & LEGAL Patient is socially appropriate for age.

HISTORY: Born in MI, moved to FL in 2008.

Education: High school completed / GED not diploma. College not completed. Real State License.

Marital status/quality of relationship: in a relationship with boyfriend for 6 years. Has a 7 year old daughter.

Support network: sister, boyfriend

Family relationships: "good with dad and sister, no good with mother".

Peer relationships: "good"

Current living conditions: Good.

Employment /occupation: Employed.

Financial means of support: Self.

Religion and spirituality (beliefs, important practices, impact on mental illness, restrictions, if any): "spiritual, not into organized religion".

Leisure and Recreational Activities: "hang out with friends, pools, dinner".

"my father was very angry all the time, I do not know if it is consider abuse". Denies trauma.

Participate in therapy in the past.

LEGAL HISTORY Denies.

OBJECTIVE:

Vital Signs: Weight: 130.00 lbs

Pt personally examined: no

Objective Notes: MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/
Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "anxious". Affect: Stable.
Thought Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and
JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies
suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language:
Intact. Orientation: Ox4. Time. Place. Person. Situation.

Short Term Memory: Intact Long Term Memory: Intact. Ability of Retrieving information and rapid forgetting.

Ability of Encoding information Intact Concentration/Attention: Intact . Judgment /Insight: Intact

ASSESSMENT:

Diagnosis: ICD-10 Codes:

1)F902; Attention-deficit hyperactivity disorder, combined

PLAN:

Procedures: 1) 000; 99205+90833

Procedure Notes: Michelle presented mildly anxious, pleasant and cooperative. Symptoms are positive for ADHD combined type.

We agreed to start Adderall 10mg daily in the am for ADHD symptoms and Trazodone 50mg 1/2 QHS PRN for sleep.

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 220761427 Encounter Date: 05/26/2020

Encounter Type: Office Visit

Medications: Adderall 10 MG Oral Tablet; Take 1 tablet orally daily in the am with food.; Qty: 30; Refills: 0
traZODone HCl 50 MG Oral Tablet; Take 0.5 tablets orally daily as needed for sleep; Qty: 15; Refills: 0

Care Plan: The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of Trazodone including but not limited to GI upset, sexual side effects, sedation, suicidal ideation, dizziness, headache and hypertension.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia, tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out the day.

Advised the patient to refrain from drug or alcohol while taking medications

Advised the patient to take the medication with food if GI upset occurs .

For any serious side effects recommended that patient stop the medication immediately.

Patient Instructions: Next Appointment: 2 Weeks.

DURATION OF VISIT: Supportive therapy was provided. Psychoeducation therapy was provided. Psychodynamic therapy was provided.

_____ Date: _____

[Provider]: Eduardo La Rosa, APRN

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 221478931 Encounter Date: 06/11/2020

Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Patient has given verbal consent that he understands the limitations of telehealth and consents to proceed with treatment through this mode

This is a 32 year old woman with a history of ADHD inattentive type. Medication Management visit.

Today Michelle reports "medication is working. When I take my medication I focused a lot better, I get done with my projects ahead of time. I am not always rushing, I actually taking the time to review my work and do not make careless mistakes as I did. I also not getting easily frustrated as I did before. My one concern the medication is effective for 2-3 hours only.

ADHD: "improving" Reports: careless mistakes and/or lack of attention to details, difficulty sustaining attention in tasks, does not seem to listen when spoken directly, failure to follow through on tasks, poor organization, losing things for task, easily distracted, forgetful in daily activities, avoids tasks that sustain/require effort, procrastinate, fidgets, squirms, leaves seat when expected to stay seated, restlessness, often on the go / acts as if driven by a motor, talks excessively, blurts out answers before questions have been completed, difficulty awaiting turn / interrupts or intrudes on others (butts into conversation or game). all symptoms are improving

ANXIETY: "significant improvement" Reports: worry, fears, apprehension, irritability, frustrated, angry, difficulty concentrating, racing and recurrent thoughts are a lot better

Sleep: "much better, I am sleeping at least 8 hours per night".

Appetite: "fine"

SUBSTANCE USE: "one glass of wine socially once per week, no nicotine, 1-2 cups of coffee daily, no illegal drugs".

Florida PMP: overdose risk score 0/999. 5/26/20 Adderall 10mg 30.

History Of Present Illness:

She works full time as property manager and real state agent. She lives with boyfriend and 7 year old daughter. Patient states that the above symptoms started since she was a child. Symptoms were manageable most of her life. She reports for the last few years symptoms are getting worse. Since April-May 2020 symptoms are worse, inability to complete tasks causes anxiety, irritability and frustrated. "lack of patient with daughter and snappy at work and I feel guilty and like I am failing them".

PAST PSYCHIATRIC HISTORY:

Past Psychiatric History: Yes. ADHD

Psych. Hospitalizations: denies

Past suicidal ideation or behavior: denies

Past violent ideations or behavior: denies

List of psychotropic medications prescribed in the past, including dosage, frequency, and side effects: denies

Substance Abuse History: Denied.

SUBSTANCE ABUSE HISTORY:

Allergies:

No known allergies

Current Medications:

none

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 221478931 Encounter Date: 06/11/2020

Encounter Type: Office Visit

Adderall 10 MG Oral Tablet; Take 1 tablet orally daily in the am with food.; Qty: 30; Refills: 0

OBJECTIVE:

Objective Notes:

MENTAL STATUS EXAMINATION Appearance: Neat. Behavior cooperative with good eye contact Gait/ Balance: Normal. Musculoskeletal: No abnormality. Speech: Regular. Mood: "good". Affect: Stable. Thought Process: Goal-directed. NO PSYCHOSIS COGNITION IS INTACT LANGUAGE, MEMORY and JUDGEMENT intact Thought Perception/Content: DENIES auditory/visual hallucinations and delusions, denies suicidal ideation intent or plan, denies homicidal ideation intent or plan. Denies violent ideation. Language: Intact. Orientation: Ox4. Time. Place. Person. Situation. Short Term Memory: Intact Long Term Memory: Intact. Ability of Retrieving information and rapid forgetting. Ability of Encoding information Intact Concentration/Attention: Intact . Judgment /Insight: Intact

ASSESSMENT:

Diagnosis:

ICD-10 Codes:

1)F902; Attention-deficit hyperactivity disorder, combined

PLAN:

Procedures:

1) 0; 99214+90833 (psychotherapy 16-37min)

Procedure Notes:

Michelle presented calm, pleasant, optimistic and cooperative. She reports ADHD symptoms have improved since starting Adderall, However, medication effects only last 2-3 hours. She has been taking 10mg BID. She also reports anxiety has greatly improved since starting Adderall".

Sleep has improved, did not start Trazodone. Has only taking Melatonin once in the last 2 weeks. We will discontinue Trazodone.

Medications:

We agreed to increase Adderall 20mg 1 tablet in the am and 1/2 tablet in the afternoon.

Adderall 20 MG Oral Tablet; Take 1.5 tablets orally daily Take 1 tablet in the am with food. Take 1/2 tablet in the afternoon.; Qty: 45; Refills: 0

Care Plan:

The patient gave informed consent for the medication regimen described in the plan of this note.

Discussed with the patient the potential side effects of STIMULANTS including decreased appetite, insomnia, tics, tachycardia, cardiac side effects, agitation, psychosis. She will stay hydrated and eat small meals thru out the day.

Advised the patient to refrain from drug or alcohol while taking medications

Advised the patient to take the medication with food if GI upset occurs .

Patient Instructions:

For any serious side effects recommended that patient stop the medication immediately.

TIME SPENT IN

Next Appointment: 1 Month.

COUNSELING:

Greater than 50% of time spent in counseling.

Counseling/psychotherapy time 16 min

Addressed patient/caregiver concerns regarding current medication regimen including effectiveness and side effects.

Addressed patient/caregiver concerns regarding diagnosis and prognosis including prognosis over time and impact of diagnosis on life functions.

Michelle Kohn

Patient ID: 99671655 DOB: 08/06/1987 Sex: F Account No.:

Encounter ID: 221478931 Encounter Date: 06/11/2020

Encounter Type: Office Visit

Patient shows understanding of counseling

Supportive therapy was provided.

Psychoeducation therapy was provided.

Psychodynamic therapy was provided.

Duration of visit: 20min.

_____ Date: _____

[Provider]: Eduardo La Rosa, APRN